

Barcode No	87943792	Lab No	16952412270003
Patient Name	Mrs.NAZIA BIBI	Reg Date	27/Dec/2024 03:31PM
Age/Sex	27 YRS/Female	Sample Coll. Date	27/Dec/2024 02:34 PM
Referred By	DR. ROBINA MIRZA	Sample Rec.Date	28/Dec/2024 08:40 AM
		Report Date	28/Dec/2024 09:40AM

HAEMATOLOGY

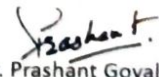
Test Name With Methodology	Result	Unit	Biological Ref.Interval
Haemoglobin HPLC			
Hb F (Foetal Hb) Level	<0.3	%	< 2.0
Peak 2	3.9	%	
Peak 3	1.6	%	<10
Hb A0	81.8	%	
HbA2	2.8	%	1.9 - 3.5
Hb D (Punjab)	0.0	%	<0.02
Hb Sickle	0.0	%	<0.02
Hb C	0.0	%	<0.02
Hb E	0.0	%	<0.02
Opinion	Normal Pattern		
Advise	Please Correlate Clinically		

Comment:

Haemoglobinopathy is a kind of genetic defect that results in abnormal structure of one of the globin chains of the hemoglobin molecule. Hemoglobin variants are abnormal forms of hemoglobin. All results have to be correlated with age and history of blood transfusion. If there is history of blood transfusion in last 3 months, repeat testing after 3 months from last date of transfusion is recommended. *In case of hemoglobinopathy, parents or family studies, DNA Analysis and Genetic counseling is advised.*

- ~ **P3 window**- Above 10% is often indicative of either denatured forms of hemoglobin (degenerated sample) or may suggest a possibility of abnormal hemoglobin variant. Hence, repeat analysis with fresh sample or DNA studies is advised.
- ~ **P2 window**- Above 10% is indicative of either glycated hemoglobin requiring correlation with diabetic status or may suggest a possibility of abnormal hemoglobin variant requiring further DNA studies for confirmation.
- ~ Screening of the spouse are strongly recommended, if pregnant woman have hemoglobinopathy.
- ~ This test detects Beta thalassemia and hemoglobinopathies. DNA analysis is recommended to rule out alpha thalassemia and silent carriers.
- ~ Mild to moderate increase in **fetal hemoglobin (HbF)** can be seen in some acquired conditions like Pregnancy, Megaloblastic anemia, Thyrotoxicosis, Hypoxic conditions, Chronic kidney disease, Recovering marrow, MDS, Aplastic anemia, PNH, Medications (Hydroxyurea, Erythropoietin, Butyrates) etc.
- ~ Iron deficiency Anemia may be associated with **low HbA2** result and may mask thalassemia condition also.
- ~ **Elevated levels of HbA2** are seen in Beta-Thalassemia, and also in other conditions like Sickle cell Alpha-Thalassemia, Megaloblastic Anemia, Unstable Hemoglobinopathies, Zidovudine- treated HIV patients, Congenital Dyserythropoietic Anemia (CDA) type 1 and Hyperthyroidism.

*Results relate only to the sample, as received.


 Dr. Prashant Goyal (DCP)
 (Director & Chief Pathologist)
 Reg. No. DMC-53016

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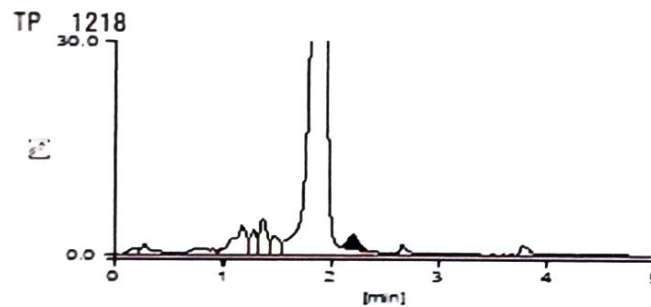
Chromatogram Report

Toosh HLC723G11 V3 07 1 2024-12-28 09:14:52
 ID 87943792
 Sample No. 2024122808570022 SL 0001 - 03
 Patient ID
 Name
 Comment

CALIB	F	$Y = 1.2255X + 0.4898$	
	A2	$Y = 1.2552X + 1.1875$	
Name	%	Time	Area
F			
A0	81.8	1.91	986.77
A2	2.8	2.21	17.35
E+			
D+			
S+			
C+			

Total Area 1206.97

HbF 0.0 % HbA2 2.8 %



[Unknown Peak]

Name	%	Time	Area
P00	0.5	0.19	6.36
P01	1.3	0.28	15.54
P02	1.3	0.78	16.19
P03	3.9	1.19	46.74
P04	1.6	1.29	19.73
P05	2.6	1.37	31.79
P06	1.7	1.48	20.21

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